

Comorbidities and Management in Psoriasis

Inflammatory Bowel Disease (IBD)

The two most common forms of IBD are **ulcerative colitis (UC)** and **Crohn's disease (CD)**. Genetic studies have identified shared risk factors between psoriasis and IBD. Polymorphisms in the gene encoding the **IL-23 receptor** have been associated with both **psoriasis** and **UC/CD**, highlighting a common immunopathogenic pathway involving the IL-23/Th17 axis.

Lymphoma and Skin Cancer

- **Lymphoma** risk is increased in patients with psoriasis. This may be due to:
 - The underlying **chronic systemic inflammation** in psoriasis.
 - Use of **systemic immunosuppressive therapies** such as **ciclosporin** and **methotrexate**, which are linked to lymphomagenesis.
- **Non-melanoma skin cancer (NMSC)**, particularly **squamous cell carcinoma (SCC)**, is also more common in psoriasis patients.
- Treatments such as **psoralen plus UV-A (PUVA)** and **ciclosporin** contribute to this increased risk.

Psychopathological Comorbidities

- Psoriasis is associated with **depression** and other **psychosocial disturbances**.
- Elevated levels of **pro-inflammatory cytokines** (e.g., TNF- α , IL-6, IL-17) involved in psoriasis pathogenesis are also implicated in the development of depression.

- Psychosocial burden often does not correlate directly with disease severity but is consistently rated by patients as one of the most distressing aspects of their condition.

Harmful Lifestyle Habits

Tobacco Use

- **Nicotine** promotes overproduction of pro-inflammatory mediators.
- Smoking is a known risk factor for immune-mediated diseases including **psoriasis, rheumatoid arthritis, IBD, and atherosclerosis**.
- It may exacerbate psoriasis and reduce treatment response.

Alcohol Consumption

- Alcohol has **immunosuppressive effects**, increasing susceptibility to infections such as **streptococcal pharyngitis**, which can trigger or worsen psoriatic flares.
- Alcohol contributes to key features of plaque psoriasis:
 - **Keratinocyte hyperplasia**
 - **Granulocyte infiltration**
 - **Dermal blood vessel dilation**
- Heavy alcohol use is linked to more severe disease and poorer treatment outcomes.

Cardiovascular Disease (CVD) and Metabolic Comorbidities

Psoriasis is independently associated with an increased risk of: - **Obesity (OB)** - **Type 2 diabetes mellitus (DM)** - **Hypertension (HT)** - **Dyslipidemia (DLP)** - **Metabolic syndrome (MS)** - **Non-alcoholic fatty liver disease (NAFLD)**

These conditions contribute to the development of **cardiovascular disease (CVD)**, the leading cause of death in industrialized countries.

Early Intervention Benefits

- Early diagnosis and management of **cardiovascular risk factors (CVRF)** can prevent or delay CVD onset.
- Prompt treatment of **psoriatic arthritis (PsA)** can slow joint damage and preserve function.

Drug Interactions and Treatment Considerations

- Some psoriasis treatments may worsen comorbidities:
- **Acitretin**: may elevate serum lipids.
- **Ciclosporin**: can increase blood pressure and impair renal function.
- Conversely, medications for comorbid conditions may influence psoriasis:
- **Beta-blockers** and **lithium** may exacerbate psoriasis.

Integrated Patient Management

Effective psoriasis care requires a **multidisciplinary, integrated approach** that includes: - Screening for and managing comorbidities. - Early referral to specialists (e.g., rheumatology, hepatology, cardiology, mental health). - Coordinating therapies to avoid adverse interactions. - Addressing lifestyle factors (smoking, alcohol, diet, physical activity).

Clinical Practice Guidance

The **Spanish Group in Psoriasis** of the **Spanish Academy of Dermatology and Venereology** has developed a clinical practice guideline to support dermatologists in the comprehensive management of psoriasis comorbidities. This summary provides a concise, office-friendly reference based on Daudén et al.'s recommendations, focusing on: - Psoriatic arthritis (PsA) -

Cardiovascular disease and risk factors (OB, DM, HT, DLP, MS) - NAFLD -
IBD - Lymphoma - Anxiety and depression - Tobacco and alcohol use

This integrated strategy aims to improve long-term outcomes and quality of life
for patients with psoriasis.